

Lesson Plan: postpartum

Programme: Registered Nursing

Year: Year 1 - Semester 1

Topic: postpartum

1. Introduction to Postpartum Period

The postpartum period is a critical time for women, often associated with significant physiological and psychological changes. It is a period when many women are at risk of mental illness due to the stress and overwhelming responsibility of caring for a new infant. Nurses play a vital role in providing high-quality postpartum care to ensure optimal health for the mother and infant.

References:

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - The time of childbirth is a period when many women are at risk of mental illness because it can sometimes be a stressful time. The added responsibility of having a young infant that needs a lot of care, can sometimes be overwhelming to a new mother.

- Source: 16. FINAL IRH I MODULE.docx - Safe motherhood encompasses a series of initiatives, practices, protocols and service delivery guidelines designed to ensure that women receive high-quality gynaecological, family planning, prenatal, delivery and postpartum care, in order to achieve optimal health for the mother, foetus and infant during pregnancy, childbirth and postpartum.

- Source: 16. FINAL IRH I MODULE.docx - We then focused the discussion on pregnancy, labour and delivery and the postpartum period. You learnt about the development of the foetus and the physiological changes a woman experiences during pregnancy.

2. Learning Outcomes

- Describe the immediate care given to a woman post-delivery.
- Describe the process of postnatal care.
- Identify and differentiate between postpartum blues, postpartum depression, and postpartum psychosis.
- Recognize the signs, causes, and predisposing factors of postpartum hemorrhage.
- Outline the general nursing responsibilities during the postpartum period.

References:

- Source: 16. FINAL IRH I MODULE.docx - Describe the immediate care given to a woman post-delivery
Describe the process of postnatal care

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - Postpartum mood symptoms are divided into 3 depending on severity. These are:

Postpartum blues

Postpartum depression

Postpartum psychosis

- Source: 22. FINAL IRH II MODULE.docx - Postpartum hemorrhage (PPH) is severe vaginal bleeding after childbirth. It's a serious condition that can lead to death. Other signs of postpartum hemorrhage are dizziness, feeling faint and blurred vision.

3. Postpartum Mood Symptoms

The postpartum period can be a stressful time, increasing the risk of mental illness. Postpartum mood symptoms are categorized into three levels of severity: postpartum blues, postpartum depression, and postpartum psychosis.

References:

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - The time of childbirth is a period when

many women are at risk of mental illness because it can sometimes be a stressful time. The added responsibility of having a young infant that needs a lot of care, can sometimes be overwhelming to a new mother. Postpartum mood symptoms are divided into 3 depending on severity. These are:

Postpartum blues

Postpartum depression

Postpartum psychosis

4. Postpartum Blues

- Timing: Occurs within 1-5 days of delivery.
- Symptoms: Emotional lability and tearfulness.
- Nature: A normal response due to hormonal changes after delivery.
- Management: Treated by assurance and given time to resolve. Strong social support (considerate husband, close relatives) to help with baby and house chores can reduce severity. Nurses should explain the physiological processes to the patient. Adequate prenatal preparation and education by midwives and nurses can prevent factors that worsen postpartum blues, such as a broken episiotomy, sore nipples, engorged breasts, and a persistently crying baby.

References:

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - This occurs within 1-5 days of delivery. Emotional lability and tearfulness is present. However, this is a normal response that occurs due to the hormonal changes that take place after delivery. It is treated by assurance and given time to resolve. Nevertheless, having a strong social support such as a considerate husband or other close relatives to help with the baby and house chores reduces the severity of the postpartum blues. You should explain to the patient the physiological processes that lead to the blues. Other postnatal difficulties which could worsen postpartum blues like a broken episiotomy, sore nipples, engorged breasts and a persistently crying baby can be prevented by adequate prenatal preparation and education by midwives and nurses.

5. Postpartum Depression (PPD)

- Timing: Occurs 2 weeks to 12 months after delivery, with symptoms potentially appearing anytime in the first year.
- Risk Factors: Higher risk for individuals with psychiatric disorders, psychosocial problems, and chronic physical illnesses.
- Symptoms: Needs verification against local guideline for specific symptoms, as the retrieved chunk states 'The symptoms include, but are not limited to the following:' without listing them.

References:

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - Postpartum depression occurs 2 weeks to 12 months after delivery. The risk is higher for people with psychiatric disorders and in individuals who have psychosocial problems as well as chronic physical illnesses.

Symptoms of PPD

What are the symptoms of post-partum depression?

It can occur anytime in the first year after delivery. The symptoms include, but are not limited to the following:

6. Postpartum Psychosis

- Timing: Most cases occur within two weeks of delivery, but can appear as early as two to three days after birth.
- Severity: Always considered a medical emergency, especially due to potential thoughts of harm.
- Prevalence: Estimated to occur in about one to two out of every 1,000 births.
- Causes: Research shows that the majority of individuals who develop postpartum

psychosis immediately after childbirth have bipolar disorder or schizoaffective disorder. However, up to half of those who experience it have no prior history of mental illness. A potential contributor is a decrease in pregnancy hormone levels (oestrogen and progesterone) postpartum, affecting mood-associated neurotransmitters like serotonin and dopamine.

- Significant Risk Factors: Previous psychotic episode, family history of bipolar disorder or postpartum psychosis, being a first-time mother, extremes of reproductive age (very young or very old), Caesarean delivery (C-section), sleep deprivation (both a risk factor and symptom), withdrawal from mood stabilizing medications, and postpartum complications with the newborn or mother.

- Symptoms/Appearance: Women may appear excited or elated, depressed, anxious, or confused, excessively irritable, or have changing moods. Noticeable and dramatic shifts in mood, similar to bipolar episodes (shifting between depression and mania), can occur over a few hours.

- Diagnosis: No set criteria; doctors ask about symptoms to determine the best course of action.

- Treatment: May include antipsychotic medications (e.g., Risperdal/risperidone, Seroquel/quetiapine, Zyprexa/olanzapine) as first-line. Lithium can reduce risks in vulnerable individuals. Electroconvulsive therapy (ECT) can rapidly decrease symptoms, including suicidal thoughts, but is not commonly used.

- Recovery: Full recovery can take up to a year, with the most severe symptoms lasting 2–12 weeks. Treatment and support are essential for recovery, though recovery is possible without them, albeit more challenging and prolonged.

References:

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - Most cases of postpartum psychosis occur within two weeks of delivery, but they can come on as soon as two to three days after giving birth. Research has shown that the majority of people who develop postpartum psychosis immediately after childbirth either have bipolar disorder, which is characterized by extreme emotional highs and lows, or schizoaffective disorder, which causes symptoms of schizophrenia and mood disorders.

Postpartum psychosis should always be considered a medical emergency, especially because symptoms may lead to thoughts of harm.

How Common Is Postpartum Psychosis?

Postpartum psychosis is estimated to occur in about one to two out of every 1,000 births.

Causes of Post-partum Psychosis

A history of bipolar disorder is the best-recognized risk factor of postpartum psychosis. However, it's been found that up to half of those who experience postpartum psychosis have no prior history of mental illness.

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - A potential contributor to the rapid mood swings seen in postpartum psychosis is a decrease in pregnancy hormone levels, including oestrogen and progesterone, which plummet in the postpartum period, affecting functions of the mood-associated neurotransmitters serotonin and dopamine.

Significant Risk Factors of Postpartum Psychosis

The risk factors include:

A previous psychotic episode

Family history of bipolar disorder or postpartum psychosis

Being a first-time mother

Extremes of reproductive age (being very young or very old)

Caesarean delivery (C-section)

Sleep deprivation, which is both a risk factor and symptom

Withdrawal from mood stabilizing medications

Postpartum complications with the newborn or mother

Women with postpartum psychosis may appear:

Excited or elated

Depressed, anxious, or confused

Excessively irritable or have a changing mood

Noticeable and dramatic shifts in mood can occur over a few hours and are similar to bipolar episodes (shifting

between depression and mania, or elevated moods and energy).

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - There are no set criteria for the diagnosis of postpartum psychosis. The doctor may ask the patient questions about her symptoms to determine the best course of action.

Treatment for postpartum psychosis may include the following:

Antipsychotic medications are the first-line treatment for psychosis, including Risperdal (risperidone), Seroquel (quetiapine), and Zyprexa (olanzapine).

Lithium is said to reduce the risks developing postpartum psychosis in people who are particularly vulnerable to postpartum psychosis.

Electroconvulsive therapy (ECT), a brief electrical stimulation of the brain, can help to rapidly decrease symptoms (including those of suicidal thoughts or behaviours), but it is not commonly used.

While it can take up to a year to fully recover from postpartum psychosis, the most severe symptoms tend to last for 2–12 weeks. Getting the right treatment and support is an essential component of recovering. Without treatment and support, recovery is still possible but can be more challenging and take longer.

7. Postpartum Hemorrhage (PPH)

- Definition: Severe vaginal bleeding after childbirth, defined as the loss of 500 ml or more of blood from the genital tract after delivery, or any amount causing sudden deterioration in the mother's condition. It is a serious, life-threatening condition.

- Timing: Can occur after delivery or up to 12 weeks postpartum. More than half of all maternal deaths occur within 24 hours of childbirth, mostly due to severe bleeding.

- Types: Primary PPH occurs within the first 24 hours of delivery (most critical period). Secondary PPH occurs from 24 hours to 6 weeks postpartum.

- Signs: Severe vaginal bleeding, dizziness, feeling faint, blurred vision, heart palpitation, feeling of weakness, and shortness of breath. These could indicate shock.

- Causes: Atonic uterus, retained placenta or fragments, tears of uterus, cervix, vagina, perineum, coagulation defects, inversion of uterus, and infection (for delayed PPH). Placenta previa also increases the risk because the lower segment of the uterus does not contract sufficiently post-delivery.

- Predisposing Factors: Maternal age 35 or over, delivery after antepartum hemorrhage (APH), multiple pregnancy, polyhydramnios, and a past history of PPH.

- Management: PPH requires prompt and effective management. Rapid action is critical for survival. Nurses must be dedicated and vigilant throughout the process of providing care to a woman during pregnancy, labour and postpartum, recognizing and diagnosing complications.

References:

- Source: 22. FINAL IRH II MODULE.docx - Postpartum hemorrhage (PPH) is severe vaginal bleeding after childbirth. It's a serious condition that can lead to death. Other signs of postpartum hemorrhage are dizziness, feeling faint and blurred vision.

PPH can occur after delivery or up to 12 weeks postpartum. More than half of all maternal deaths occur within 24 hours of childbirth, mostly due to severe bleeding. PPH accounts for about 25% of maternal deaths worldwide. Rapid action is critical for survival.

Causes of Postpartum Haemorrhage

The causes of PPH are as follows:

Atonic uterus

Retained placenta or fragments

Tears of uterus, cervix, vagina, perineum

Coagulation defects

Inversion of uterus

Infection (delayed PPH)

Pre-disposing Factors of Postpartum Haemorrhage

Maternal age 35 or over

Delivery after APH

Multiple pregnancy

Polyhydramnios

Past history of PPH

Do not ignore any of the patient's complaints about dizziness, excessive bleeding, heart palpitation, feeling of weakness and shortness of breath. These could be indications for shock as a result of excessive bleeding.

Management of Postpartum Haemorrhage

PPH is a life-threatening complication that requires prompt and effective management.

- Source: 9. FINAL PUBLIC HEALTH NURSING I.docx - Postpartum Haemorrhage

This is the loss of 500 ml or more of blood from the genital tract after the delivery of the baby, or any amount that brings about sudden deterioration in the mother's condition.

There are two types of postpartum haemorrhage:

Primary postpartum haemorrhage (PPH) occurs within the first 24 hours of delivery, which is the most critical period.

Secondary PPH occurs anywhere from 24 hours to 6 weeks postpartum.

- Source: 22. FINAL IRH II MODULE.docx - After the baby is delivered, observe the mother for postpartum haemorrhage because the lower segment of the uterus does not contract and retract to control haemorrhage.

Complications of Placenta Praevia

Maternal

Antepartum haemorrhage

Malpresentation

Abnormal placentation

Postpartum haemorrhage

Placenta praevia increases the risk of puerperal sepsis and postpartum haemorrhage because the lower segment to which the placenta was attached does not contract sufficiently post-delivery.

8. Nursing Responsibilities in Postpartum Care

- Provide immediate care to the woman post-delivery.
- Implement and monitor postnatal care processes.
- Educate patients on physiological processes leading to postpartum blues and strategies to prevent worsening factors (e.g., episiotomy care, nipple care, breast engorgement management, coping with a crying baby).
- Assess for early indicators of mental health disorders, including postpartum blues, depression, and psychosis.
- Be vigilant for signs of postpartum hemorrhage and initiate prompt, effective management.
- Provide supportive care, including bed baths, oral care, and feeding, as needed.
- Ensure high-quality postpartum care as part of safe motherhood initiatives.

References:

*- Source: 16. FINAL IRH I MODULE.docx - Describe the immediate care given to a woman post-delivery
Describe the process of postnatal care*

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - You should explain to the patient the physiological processes that lead to the blues. Other postnatal difficulties which could worsen postpartum blues like a broken episiotomy, sore nipples, engorged breasts and a persistently crying baby can be prevented by adequate prenatal preparation and education by midwives and nurses.

- Source: 22. FINAL IRH II MODULE.docx - Rapid action is critical for survival.

Management of Postpartum Haemorrhage

PPH is a life-threatening complication that requires prompt and effective management.

9. Conclusion

The postpartum period is a dynamic phase requiring comprehensive nursing care. Understanding the physiological and psychological changes, including postpartum mood symptoms and the risks of postpartum hemorrhage, is crucial for nurses. Vigilance, early recognition, and appropriate management are key to ensuring the well-being of the mother and infant during this critical time.

References:

- Source: 16. FINAL IRH I MODULE.docx - We then focused the discussion on pregnancy, labour and delivery and the postpartum period. You learnt about the development of the foetus and the physiological changes a woman experiences during pregnancy.

- Source: 18. FINAL PSCHIATRY & MENTAL HEALTH NURSING I.docx - Postpartum mood symptoms are divided into 3 depending on severity. These are:

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